

CRECOOLT Manuscript — Pre-Submission Fix Checklist

Novel vs. older regimens for CRE infection in liver transplant recipients

2026-07-20

Items to resolve before submitting to CID / CMI, from the statistical reproduction and review (line numbers refer to the manuscript draft; see [STATUS.md](#) for full detail).

Major — must fix

- [?] **Mortality HR precision.** Reported 0.58 (95% CI 0.39–0.86, $p=0.007$) is the *full-cohort 9-covariate* value. The described **era-restricted** model (N=182, 4-covariate PS) gives **0.58 (95% CI 0.34–0.99, $p=0.044$)**. Report the era-restricted CI/p. (*Abstract L70–71, L82; Results L162.*) Figure 1 has been updated to match.
- [?] **Crude mortality comparator is cross-era.** “45.6% vs 31.2%” pairs full-cohort old (mostly pre-2018) with the post-2018 novel arm. Use the within-era crude (**35.6% vs 31.2%**) or drop the crude comparator for the within-era HR. (*Abstract L69–70, L81–82; Results L154–155.*)
- [?] **CID abstract percentages are wrong.** “15.6% vs 31.8%” are the **30-day** figures under a 90-day mortality claim (and disagree with the CMI abstract). Correct to the 90-day within-era values. (*L81–82.*)
- [?] **“Reducing mortality by 50%” overstates the effect.** HR 0.58 $\approx$ 42% relative reduction, and borderline ($p=0.044$) in the analysis as described. Soften. (*Discussion L168–169.*)
- [?] **Supplementary Figure 1 (Love plot).** Regenerate to show the 4 covariates actually used (age, MELD, multisite colonisation, SOFA), era-restricted — not the old 9-covariate full-cohort version. (*L156–159; use [loveplot_era](#).*)

Moderate

- [?] **Unsupported subgroup claim.** Discussion states “no significant interactions across subgroups,” but no subgroup analysis appears in Methods/Results. Add the forest plot as a supplementary figure ([figure_forest_90d](#)) or delete the sentence. (*L180–181.*)
- [?] **Add the ascertainment limitation.** Pre-2018 records show 0 resistance events, ~0 recurrence, no clearance dates, and outcomes coded “No” rather than missing — so the crude cross-era recurrence/resistance rates cited (L188–192) are partly artifactual. State this. (*Limitations L196–202.*)
- [?] **“N=182 events” → “N=182 patients.”** 182 is the era cohort size; 90-day deaths = 59. (*L161.*)
- [?] **Complete the statistics-software sentence** (currently “...performed using”): e.g., R v4.5.1 with [survival](#) and [tidyverse](#). (*L142.*)

Minor / typographical

- [?] “0.52-2-05” → “0.52-2.05” (*L72, L83, L166.*)
- [?] “2018-2015 cohort” → “2018-2025” (*L137.*)
- [?] “resistance emerge” → “resistance emergence” (*L71, L76, L122, L164.*)
- [?] Label the secondary estimates as **subdistribution HR (sHR)**, not HR (*Abstract L71–72; Results L165.*)

Journal formatting

- [?] **Word count 2656** exceeds CID (1500) and CMI (1400) limits — substantial trimming required.
- [?] **CID abstract** exceeds the 50-word limit; choose the target journal and format accordingly.

- **Figure legends.** Ensure legends exist for Figure 1 (mortality), Suppl. Fig 1 (era Love plot), Suppl. Fig 2 (recurrence/resistance CIF); confirm the old prophylaxis-paper Figure 1 legend is gone.
- Fill in **Keywords** (L44) and **corresponding author** (L47).

Verified correct — no action needed

- Recurrence/resistance subdistribution HRs (1.03 and 1.45) reproduce; comparable between groups.
- Fine-Gray correctly reserved for the secondary endpoints (not applied to all-cause mortality, where nothing competes with death).
- Positivity / era-restriction rationale is well stated (*L135–137, L199–202*).